National Cancer Control Programme (NCCP) 00790 · clinician draft

TRAIN-2 with Phesgo

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin combination

Active medicines: Paclitaxel + Carboplatin + Pertuzumab + Trastuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before paclitaxel.
- Day 8 and thereafter: no routine dexamethasone; chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before. Consultant may add steroid for risk/prior reaction or omit famotidine if appropriate; monitor closely.
- Rechallenge steroid changes require consultant guidance; consider analgesia for taxane aches and granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk.
- Day 1 antiemetic steroid is reconciled with paclitaxel premedication; do not add two steroid courses.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00790 v3 · p12 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.15 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · paclitaxel only

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No carboplatin or Phesgo on Day 8; no planned Day 15 paclitaxel.
- Day 8 and thereafter: no routine dexamethasone; chlorphenamine 10 mg intravenous (IV) and famotidine 20 mg IV 30 minutes before. Consultant may add steroid for risk/prior reaction or omit famotidine if appropriate; monitor closely.
- Rechallenge steroid changes require consultant guidance; consider analgesia for taxane aches and granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00790 v3 · p12 · NCCP antiemetics v8, Table 1 pp4–5

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Postoperative HER2-only phase · confirm selected regimen

Active medicines: Selected postoperative HER2 medicine; separate source regimen

Emetogenic risk: Confirm the selected postoperative regimen

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Open the separately selected postoperative regimen before issuing supportive medicines. Its emetic risk, premedication and tests apply.
- Phesgo or trastuzumab alone generally needs no routine antiemetic prophylaxis. Do not apply this statement automatically to an antibody-drug conjugate.
- Pathology, including residual invasive disease, informs the next HER2 medicine and remaining treatment course.
- Continue cardiac monitoring on the selected treatment plan; surgery does not restart the clock.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00790 v3 · p1 · NCCP antiemetics v8, Table 1 pp4–5

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